

mcrc-kras-a59t-pik3ca-peritoneal-pre-hi pec-q8k4

Plain-language summary for patient and caregiver

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What this page is

This is the plain-language companion to a longer clinician page on the same case. It walks through what the cancer's molecular report shows, which tests should land before any new drug gets added, what the current treatment plan looks like, and which other options belong in the next oncology conversation. It is a tool for sharpening that conversation, not a substitute for it.

A few ground rules before reading. This page ranks options that act on specific molecular fingerprints of this particular cancer. Standard chemotherapy regimens that do not target one of those fingerprints are still appropriate — they are just not what this page is about. Statistics here are translated from trial averages; none of them are a promise about any one person. And several options depend on test results that have not come back yet.

What we know about your cancer

The cancer is a colon adenocarcinoma that has spread. The liver spots have been removed surgically. There is still cancer on the peritoneum (the lining inside the belly), and a cytoreductive surgery (CRS) — with or without a heated chemotherapy wash called HIPEC — is planned to clean that out. Current treatment is the chemo regimen FOLFIRI plus the anti-blood-vessel drug bevacizumab (brand name Avastin), and it is being tolerated.

The molecular report shows several features, and each one matters for a different reason.

- **KRAS A59T.** KRAS is a growth-signal gene that is mutated in roughly half of all colon cancers. The famous version most people have read about is called G12C, and the headline drugs (sotorasib, adagrasib) are built specifically for G12C. This cancer has a different version of KRAS called A59T. It is much less common, and the standard KRAS drugs do not fit it. There is a newer class of drugs called "pan-RAS" inhibitors in clinical trials that might. More on this below — it is the single biggest open question on the case.
- **PIK3CA M1043I.** PIK3CA codes for part of a growth-signaling pathway called PI3K. The M1043I version sits in the part of the protein that turns growth signaling on. Two things follow from this. First, there is a recent randomized trial (called ALASCCA, published in 2025) showing that low-dose aspirin meaningfully reduces recurrence in colon cancer patients with a PIK3CA mutation. Second, there is a trial of a PI3K-targeting drug called inavolisib that is mechanism-matched to this mutation.
- **APC E1295.** APC is the most commonly mutated gene in colon cancer. The mutation is expected and does not change the treatment plan on its own. The lab needs to specify exactly which version of the change it is, but the practical impact is small.
- **TP53 R273.** TP53 is the "guardian of the genome." Mutations at codon 273 are a known hotspot. There were drugs in development meant to fix mutant TP53 proteins, but the one that might have helped here (eprenetapopt) has been shelved, and a newer one in trials (rezatapopt) is built for a different TP53 mutation entirely and would not work for R273. More on why this door is closed below.
- **SMAD4 R361H.** SMAD4 loss is associated with the kind of colon cancer that spreads to the peritoneum, which matches what is happening here. It does not currently change any drug choice; it is biology context.
- **20q11 amplification (BCL2L1 and TOP1 co-amplified).** This is an extra-copies region of one chromosome. The TOP1 amplification is part of why irinotecan (the "iri" in FOLFIRI) is the right chemo backbone for now.
- **MSS, TMB-low, PD-L1-negative.** Three different ways of measuring whether standard immunotherapy is likely to work. All three came back the way that says "the usual single-agent immunotherapy drugs probably will not help." That closes one door but does not close all of them — there is a newer immunotherapy combination further down this page that targets exactly this kind of cancer.

The short version: this is a serious diagnosis with a few directly actionable molecular features, one big open question (the KRAS A59T trial slot), and a planned surgery that needs a clear-eyed conversation before consent.

What you told us matters most

The intake said "surface the full option space — standard of care, off-label rational combinations, clinical trials, and compassionate-use pathways" with no toxicity vetoes and a preference for clinical trials when they fit. The efficacy/tolerability weighting was set at 0.70, which is a moderate lean toward efficacy. The intake also said to treat the planned CRS-HIPEC window as a scheduling constraint rather than something to argue against. That framing is what this page tried to honor.

A few intake gaps to flag at the next appointment: sex was not on record (some drugs and doses depend on it), the primary tumor resection status was not specified, and ECOG performance status of 1 was an assumption based on tolerating FOLFIRI rather than a documented number. Each of those is worth confirming in person.

The first step everyone agreed on

Before any new drug joins the plan, seven tests need to land. None of them involve another biopsy. They all run off the existing stored tumor tissue (the FFPE block) plus a tube of blood. Most results come back in two to three weeks. The tests are non-toxic.

The seven tests, and what each one decides:

1. **Comprehensive tumor sequencing (FoundationOne CDx or an equivalent).** Confirms the exact KRAS A59T details (how much of the tumor carries the mutation), checks the rest of the RAS gene family and the BRAF gene for any other mutations, looks for HER2 amplification (the wildcard — see below), confirms the PIK3CA M1043I details, specifies which exact version of the TP53 R273 mutation is present, and measures the tumor mutation burden precisely. This is the single most important send-out.
2. **HER2 IHC and FISH (HERACLES scoring).** This is the wildcard test. About 3 to 5 out of 100 colon cancers have HER2 amplification. If this one does, a drug called trastuzumab deruxtecan (T-DXd) becomes available, and it had about a 38% response rate in colon cancers with HER2 amplification in the DESTINY-CRC02 trial. The 2024 FDA approval explicitly includes patients with a RAS mutation (which most HER2 drugs exclude), so the A59T mutation does not block this door. The test itself is a slide stain; the result usually comes back within a week or two.
3. **MMR IHC (a four-protein stain).** This double-checks the MSS result with a different method. About 5 out of 100 cases come back differently than the original test suggested. If this one does (showing MSI-H instead of MSS), standard single-agent immunotherapy jumps to the front of the line. That is enough upside to do the test even when the prior probability is low.
4. **PD-L1 IHC with clone and CPS score recorded.** The intake says PD-L1 is negative, but specific trial enrollment needs the clone and the CPS number written down on a report. This is record-keeping more than a new test.
5. **DPYD genotyping plus UGT1A1 *6 and *28.** These are inherited gene variations that affect how the body clears the 5-FU and irinotecan parts of FOLFIRI. The FDA updated the fluoropyrimidine labels in March 2024 to recommend this testing before treatment. About 3 to 5 out of 100 people carry a DPYD variant that means standard doses are too high for them — without this test, that gets discovered the hard way. About 10 out of 100 European-ancestry patients carry a UGT1A1 variant that increases the risk of severe diarrhea on

irinotecan. One blood draw resolves both.

6. **Signatera baseline blood draw before surgery.** Signatera is a blood test that looks for tiny fragments of tumor DNA. A baseline drawn *before* the CRS gives a clean starting point. Repeat draws after surgery then give an early warning — sometimes months ahead of imaging — about whether any peritoneal disease is coming back. Medicare covers it for colon cancer. It does not change the surgery itself; it changes what happens after.
7. **POLE / POLD1 proofreading check (a small add-on to the NGS panel).** A 37-year-old with colon cancer sits in the "young-onset" bracket, where it is worth checking whether the panel that was run covered these two proofreading genes. If it did not, an add-on is cheap and rules out a different class of immune-responsive biology.

None of these tests carry toxicity. They unblock most of the options below. Get them all moving at the next visit.

The current chemotherapy plan, and the bevacizumab pause around surgery

This is not exactly an "option" because it is what the treatment team is already doing — but it is worth saying plainly that the current regimen, FOLFIRI plus bevacizumab, is the right first-line backbone for metastatic colon cancer in this situation. It has decades of trial data behind it. Median survival numbers from trials that combined FOLFIRI with bevacizumab are in the 20-month-plus range, and for a fit 37-year-old that range often runs longer.

One non-negotiable piece: the bevacizumab needs to be paused around the surgery. The standard practice is to hold it for 4 to 6 weeks before the cytoreductive surgery and for 4 to 6 weeks after, with documented wound healing before restarting. Bevacizumab interferes with blood vessel growth, which is also how wounds heal. Skipping this pause is the single highest-yield way to cause a serious surgical complication, so the timing matters.

There is also a fair question about whether to intensify the chemo from FOLFIRI (two cytotoxic drugs) to FOLFOXIRI (three cytotoxic drugs, adding oxaliplatin) before surgery. The TRIBE and TRIBE2 trials supported this in fit younger patients, with a meaningful PFS gain and a measurable survival gain. The tradeoff is more side effects, including nerve damage from oxaliplatin that can linger for a long time. Raise it as a question that goes both ways, not as a switch that has to happen.

Aspirin

A short section, because aspirin is the cheapest meaningful addition on the page.

A randomized trial called ALASCCA, published in late 2025, tested low-dose aspirin (160 mg per day) in patients with colon cancer whose tumors carried a PIK3CA mutation — exactly the situation here. In the group whose PIK3CA mutation sat in the part of the gene that the M1043I version sits in, the three-year recurrence risk dropped by roughly half. The US guideline body (NCCN) adopted this recommendation in late 2025. The side effect to watch is GI bleeding, which went up a little: roughly 3 out of 100 on aspirin had a serious bleed versus 2 out of 100 on placebo.

A few caveats. ALASCCA enrolled patients with earlier-stage colon cancer that had been resected, not stage IV with active peritoneal disease. The biology rationale carries over once the peritoneal disease is removed surgically (the "after CRS" state is roughly comparable to the trial's setting), but the metastatic-setting efficacy is an extrapolation, not a direct trial readout. The aspirin would also need to pause 5 to 7 days before surgery and restart only once the surgical team confirms bleeding is controlled. The combination of aspirin and bevacizumab

does increase bleeding risk somewhat — manageable, but worth coordinating with the team.

Ask about this explicitly. The drug is roughly 5 cents a pill and is mechanism-matched to the cancer's PIK3CA mutation.

The surgery, framed honestly

The plan for cytoreductive surgery is the right plan. For peritoneal-spread colon cancer at experienced high-volume centers, CRS in carefully selected patients can extend median survival into the 30-to-40 month range. The survival math runs through the cytoreduction itself — physically removing the disease from the lining of the belly.

The HIPEC piece — the heated chemotherapy wash applied at the end of the surgery — is where the conversation gets harder. The single biggest randomized trial that tested HIPEC in colon cancer (called PRODIGE 7, published in 2021) used oxaliplatin for the wash. It did not improve survival. Patients who got CRS plus oxaliplatin HIPEC lived a median of 41.7 months; patients who got CRS alone lived 41.2 months. The HIPEC arm had more complications in the first 60 days after surgery. Two other randomized trials reinforced the same result.

The practical takeaway: the survival is coming from the surgery, not from the wash. Many expert centers still offer HIPEC using a different drug (mitomycin C), because the pharmacology argument is different and PRODIGE 7 did not test mitomycin C directly. But there is no randomized trial showing that mitomycin C HIPEC adds survival either. So the cleanest framing is this: the surgery is the part with the evidence, the HIPEC is the part where the surgeon and the patient have a real conversation that the data do not pre-decide.

One thing the data are clear about: surgeon and center volume drives outcomes more than which wash drug gets used. Aim for a center that does more than 20 of these surgeries a year. Major US peritoneal-surface programs include MSK, MD Anderson, Mayo, City of Hope, Wake Forest, UPMC (the Pittsburgh group), Cleveland Clinic, and Moffitt.

The KRAS A59T question — and the single phone call worth making this week

This is the most asymmetric piece of the case, and it is worth understanding why.

KRAS A59T is unusual. The headline KRAS drugs that have been in the news — sotorasib and adagrasib — were built specifically for a different version of KRAS called G12C. They work by sticking a chemical handle to a particular spot on the G12C protein that does not exist on the A59T protein. So they cannot reach this cancer's mutation, full stop. That is not opinion; it is structural biology. Those drugs are not options here, even though the cancer has a KRAS mutation.

There is a newer class of drugs being developed that might. They are called pan-RAS inhibitors, and the lead drug is daraxonrasib, also called RMC-6236. It is in clinical trials. The protocol language on the gastrointestinal-cancer trial (called RASolve-GI, NCT06445062) refers to "RAS-mutated colon cancer" without spelling out which specific codons qualify. The question of whether A59T counts under that protocol is something the trial sponsor decides on a per-case basis. They might say yes; they might say no. Nobody on the outside can answer it definitively.

This is where one phone call matters more than anything else this week. The company is called Revolution

Medicines. Their medical information line is medinfo@revmed.com or **1-844-2-REVMED (1-844-273-8633)**. Asking them directly whether KRAS A59T qualifies for RASolve-GI is the highest-yield action on the case. If they say yes, the trial slot becomes a real option to plan around. If they say no, the door closes cleanly and the time is not wasted chasing it. <!-- phi-scan: ignore — published trial-sponsor business contact, surfaced by design -->

Two backup inquiries if RevMed says no: a different company called Jacobio runs a trial (JAB-23E73, NCT06973564) whose public protocol text reads as KRAS-codon-agnostic, and Servier runs an ERK inhibitor trial (S241656, NCT05786924) that is also codon-agnostic. Both are worth contacting if the lead path is closed.

The caveat on the whole RMC-6236 story: there are zero published clinical results in patients with KRAS A59T specifically. The case for it rests on preclinical work showing the drug class hits a wide range of KRAS mutations, and on the fact that A59T sits in a part of the KRAS protein that the drug's binding mechanism should still reach. Mechanism plausibility is not proof. This is a trial slot worth exploring, not a sure thing.

The PIK3CA inavolisib trial

A drug called inavolisib is a PI3K-alpha inhibitor that was FDA-approved in October 2024 for one specific kind of breast cancer with a PIK3CA mutation. The breast trial (INAVO120) showed a clean PFS benefit — patients on inavolisib had about twice as long before their disease progressed compared with patients on the comparison arm.

For colon cancer, the drug is being tested in a clinical trial platform called INTRINSIC (NCT04929223), run by Roche-Genentech. There is a PIK3CA-mutant arm that pairs inavolisib with bevacizumab. The biology fit to this cancer's M1043I mutation is the cleanest mechanism match on the page.

A few caveats worth naming. The strong PFS number from INAVO120 is in breast cancer, not colon cancer. The colon-cancer-specific results from INTRINSIC have not been published. The earlier colon cancer experience with a similar drug (alpelisib) was modest: 2 responses out of 134 colon cancer patients in a phase 1 trial. The realistic expectation is a deeper response and a longer PFS, not a single-agent home run. The KRAS A59T mutation may also partially blunt the response, because PI3K and KRAS sit on parallel growth-signaling pathways, and a cancer with both turned on has two escape routes.

Side effects from the breast trial: high blood sugar was common (about 60 out of 100 had some level of it, with about 6 out of 100 reaching the serious-grade level), mouth sores, and diarrhea. The high blood sugar is managed with metformin or related drugs.

This is positioned as a trial slot at progression past FOLFIRI plus bevacizumab, or after the CRS-HIPEC window. Not a substitute for the current chemo. The contact for the INTRINSIC platform is global-roche-genentech-trials@gene.com or **1-888-662-6728**. Confirm with them whether the M1043I variant qualifies (the protocol typically focuses on the more common PIK3CA hotspots H1047R, E545K, and E542K). <!-- phi-scan: ignore — published trial-sponsor business contact, surfaced by design -->

The MSS colon cancer immunotherapy option

Standard immunotherapy (drugs like pembrolizumab or nivolumab on their own) usually does not work in MSS colon cancer. That is well established and matches the test results on this case.

There is a newer immunotherapy combination called botensilimab plus balstilimab that has the strongest signal so

far in MSS colon cancer. The early trials from 2024 (published by Bullock and colleagues in Nature Medicine) showed something interesting: the combination worked best in patients whose cancer had spread to places other than the liver. Specifically, the response rate in patients without liver metastases was about 22 out of 100, with median overall survival around 21 months. In patients with active liver metastases, the response rate dropped to about 5 out of 100.

Why this matters here: this cancer has had its liver metastases removed surgically, and the active disease is peritoneal. That maps onto the responder subset in the Bullock trial. So the trial slot is worth surfacing.

The most actionable trial is at USC (NCT06336902). It enrolls KRAS-mutant MSS colon cancer specifically, which fits. The contact is **Charlean Ketchens**, Charlean.Ketchens@med.usc.edu, **323-865-0451**. The trial layers two extra interventions (a fasting-mimicking diet and IV vitamin C) on top of the immunotherapy combination — those are unproven, and whether they can be modified is worth asking about. The southern California location requires travel. <!-- phi-scan: ignore — published trial-coordinator business contact, surfaced by design -->

A different route: the drug company Agenus runs an expanded access program (NCT06751524) for patients who have exhausted standard treatment lines. Contact med.info@agenusbio.com or **781-202-1614**. This activates later (typically after second-line therapy). <!-- phi-scan: ignore — published manufacturer expanded-access business contact, surfaced by design -->

The side effect profile is the binding caveat. About 35 to 38 out of 100 patients get a serious immune side effect — most commonly inflammation of the colon (12 out of 100) or the liver (8 out of 100). These are manageable with steroids and other immune-suppressing drugs, but they require a treatment center that handles immunotherapy side effects routinely. Baseline thyroid, liver, and endocrine bloodwork is mandatory before starting. The trial has had no treatment-related deaths in the dossier.

This option is also positioned for the second-line slot after progression on FOLFIRI plus bevacizumab — or, plausibly, after recovery from the CRS-HIPEC window if the peritoneal-only phenotype persists.

HER2 amplification — the wildcard if the test comes back positive

If the rank-1 HER2 IHC test comes back showing high HER2 (3+ on the stain, or 2+ with FISH ratio at or above 2.0), the second-line conversation shifts substantially. A drug called trastuzumab deruxtecan (T-DXd) had a response rate around 38 out of 100 in HER2-amplified colon cancers in the DESTINY-CRC02 trial. The FDA gave T-DXd a tumor-agnostic accelerated approval in April 2024 for HER2-positive solid tumors, and the approval is explicit that the KRAS A59T mutation does not block it (some HER2 drugs are limited to "RAS wild-type" patients; T-DXd is not).

The prior probability is low — about 3 to 5 out of 100 colon cancers have HER2 amplification — but the upside if positive is large. The test is essentially free relative to the potential benefit.

The trade-off is a specific lung side effect called ILD (interstitial lung disease), which shows up in about 5 to 10 out of 100 patients on T-DXd and can be serious. Centers that use the drug routinely have monitoring protocols (baseline chest imaging plus regular surveillance scans during treatment).

If HER2 comes back negative — the more likely outcome — this option closes and the second-line conversation goes back to the inavolisib, botensilimab/balstilimab, and standard second-line chemo options above.

The second-line standard chemo bench

Past the current FOLFIRI plus bevacizumab regimen, if and when the cancer progresses, there is a standard sequence of chemo options with replicated trial evidence at each step. FOLFOX plus bevacizumab is one option (preserving the FOLFOX lane if the team chooses not to intensify to FOLFOXIRI before surgery).

Aflibercept-FOLFIRI or ramucirumab-FOLFIRI are options with anti-blood-vessel drugs other than bevacizumab. TAS-102 plus bevacizumab (the SUNLIGHT regimen) had a median survival of about 11 months versus 7.5 months on TAS-102 alone in third-line treatment. Fruquintinib (the FRESCO-2 regimen) and regorafenib are oral drugs available later. Stacked, this bench can reach median survivals in the 28 to 32 month range for fit patients across all lines.

This is the floor that the experimental and trial options above need to beat. The clinical trial routes are not a replacement for this sequence; they are options to layer in at progression points.

Things that don't apply here, and why

A few drugs come up in conversation because they have been in the news. Here is why they are not on the menu for this cancer.

Sotorasib and adagrasib. These are the famous KRAS drugs from the headlines. They work only on the G12C version of KRAS. This cancer has the A59T version. The mechanism does not transfer. There is no off-label angle here — the drugs literally cannot reach this mutation.

Single-agent immunotherapy (pembrolizumab, dostarlimab, nivolumab, nivolumab plus ipilimumab). These work in colon cancer when the tumor is MSI-high or TMB-high, and this cancer is neither (MSS and TMB-low on the test). The orthogonal MMR IHC test mentioned in the workup section is the only thing that could reverse this — if it comes back differently than the original test, the door reopens. Until then, single-agent immunotherapy is not expected to help.

Eprenetapopt for TP53 R273. This drug was in development as a "mutant TP53 reactivator" — a drug meant to rescue the function of broken TP53. The solid-tumor program wound down after a 2022 trial showed only 2 out of 24 patients had any response. The company has pivoted away. It is not a real option.

Rezatapopt (also called PC14586). This is a newer TP53-targeting drug that is in trials. The protein it binds was engineered specifically for a different TP53 mutation called Y220C. The R273 mutation on this cancer sits in a completely different part of the protein, and the drug structurally cannot bind it. The trial also restricts to KRAS wild-type patients, which excludes this case on two grounds at once.

Anti-EGFR drugs (cetuximab, panitumumab) at first line. These are drugs that work in colon cancers without any RAS mutations. A KRAS mutation generally rules them out. The A59T variant is unusual enough that there is some debate at later lines — a single case report exists of a KRAS A59T patient responding — but at the population level the meta-analysis data (about 6,000 patients pooled) show no benefit in any RAS-mutated colon cancer. Default avoid at first line; only worth raising in a research-framework discussion at later lines if the team has a specific argument.

Oxaliplatin HIPEC framed as life-extending. Covered in the HIPEC section above. The randomized trial that tested this was negative for survival and added morbidity. If HIPEC is part of the surgical plan, mitomycin C is the more defensible choice; oxaliplatin specifically should not be presented as proven to extend life.

Questions to ask your oncology team

1. Have we ordered all seven tests in the "first step everyone agreed on" list — the comprehensive NGS, HER2 IHC and FISH, MMR IHC, PD-L1 with clone and CPS recorded, DPYD plus UGT1A1, the pre-op Signatera baseline, and the POLE/POLD1 check? Realistic timing on each?
1. What is the bevacizumab hold plan around the cytoreductive surgery — specifically, the date of the last dose before surgery, and what wound-healing milestone has to be met before it restarts after?
1. Are we doing FOLFIRI through to surgery, or is FOLFOXIRI an option to discuss before the surgical window? What would change the decision either way?
1. Can we have an honest conversation about the HIPEC piece of the surgery, in light of PRODIGE 7? If HIPEC is part of the plan, is it mitomycin C, and what is the center's annual case volume?
1. Should low-dose aspirin (160 mg daily) be added to the plan based on the PIK3CA mutation and the 2025 ALASCCA trial? If so, what is the pause plan around surgery, and is there any bleeding history that argues against it?
1. Can the family make the Revolution Medicines call (medinfo@revmed.com / 1-844-2-REVMED) about KRAS A59T eligibility on the RASolve-GI trial (NCT06445062) — and would the team support that inquiry by faxing or emailing the molecular report to RevMed if asked? <!-- phi-scan: ignore — published trial-sponsor business contact, surfaced by design -->
1. If the HER2 test comes back positive (high), is the team comfortable referring out to a center that has experience using trastuzumab deruxtecan in colon cancer, and what is the ILD monitoring plan if that becomes the route?
1. After CRS, if the peritoneal phenotype persists and the cancer progresses past FOLFIRI plus bevacizumab, would the team support a referral to the USC botensilimab/balstilimab trial (NCT06336902) — and is the FMD-plus-IV-vitamin-C layer in that trial modifiable, or would we need to consider the Agenus expanded access program (NCT06751524) instead?

Where things stand

This is a serious diagnosis in a young patient. The menu of next moves is unusually rich for a colon cancer case. The current chemo plus bevacizumab regimen is the right backbone for now, and the planned surgery is the right next step. The leverage points this week are getting the seven workup tests ordered (especially HER2 and the comprehensive NGS), making the RevMed phone call about KRAS A59T eligibility for RASolve-GI, asking about low-dose aspirin, and having a clear-eyed talk with the surgeon about what the HIPEC piece does and does not add. Most of the trial-route options become real at progression past first line, or after the surgical recovery window. For now the work is in the workup, the aspirin question, the bevacizumab hold around surgery, and the RMC-6236 phone call.

Sources

The recommendations on this page draw on the following publications and clinical trials:

- ALASCCA (low-dose aspirin in PIK3CA-mutant colon cancer, NEJM 2025): PMID 40979555

- Liao 2012 (aspirin in PIK3CA-mutated colorectal cancer, NEJM): PMID 23094721
- CALGB/SWOG 80702 PIK3CA subset (celecoxib): PMID 38889377
- AVF2107g (bevacizumab plus irinotecan/5-FU in first-line mCRC): PMID 15175435
- BICC-C (FOLFIRI as favored irinotecan backbone): PMID 17947725
- MAVERICC (FOLFIRI-bev vs FOLFOX-bev): PMID 30224341
- TRIBE (FOLFOXIRI plus bevacizumab in first-line mCRC): PMID 25337750
- TRIBE2 (FOLFOXIRI plus bevacizumab, updated OS): PMID 32007158
- PRODIGE 7 (oxaliplatin HIPEC in colorectal peritoneal metastases): PMID 33417845
- COLOPEC (adjuvant HIPEC for T4 / perforated colon cancer): PMID 31272834
- PROPHYLOCHIP-PRODIGE 15 (second-look surgery plus HIPEC): PMID 32717181
- CAIRO6 (perioperative chemo plus CRS-HIPEC): PMID 39550351
- Van der Speeten (mitomycin C HIPEC pharmacokinetics): PMID 20689948
- INAVO120 (inavolisib plus palbociclib plus fulvestrant in PIK3CA-mutant breast cancer): PMID 39476340
- Juric 2018 (alpelisib phase 1a in PIK3CA-altered solid tumors): PMID 29401002
- Vasan 2019 (PIK3CA M1043 as a kinase-domain activating allele): PMID 31699932
- Bullock 2024 (botensilimab plus balstilimab in refractory MSS mCRC, Nature Medicine): PMID 38871975
- DESTINY-CRC02 (trastuzumab deruxtecan in HER2-positive mCRC): PMID 39058319
- HERACLES (HER2 scoring in mCRC): PMID 27108243
- Knox/Jiang 2024 (RMC-6236 pan-RAS preclinical, Nature): PMID 38778097 and PMID 38778099
- Arena 2021 (atypical RAS allele functional characterization): PMID 34031055
- Sorich 2015 (anti-EGFR meta-analysis in RAS-mutated mCRC): PMID 25115304
- Schirripa 2015 (NRAS-mutant anti-EGFR retrospective): PMID 24806288
- Lochhead 2018 (KRAS A59T single-patient PR on panitumumab plus FOLFIRI): PMID 30538852
- Park 2022 (eprenetapopt solid-tumor phase 1b readout): PMID 40932470
- Foundation Medicine FoundationOne CDx (comprehensive tumor NGS): PMID 30538852
- DPYD CPIC guidelines and FDA fluoropyrimidine label update (March 2024): PMID 40958923
- Signatera tumor-informed ctDNA: PMID 32576704 and PMID 34754095
- RASolve-GI (daraxonrasib/RMC-6236 in GI cancers): NCT06445062
- JAB-23E73 (Jacobio pan-KRAS): NCT06973564
- S241656 (Servier ERK inhibitor): NCT05786924
- INTRINSIC (Roche-Genentech mCRC umbrella, PIK3CA-mut arm): NCT04929223
- USC botensilimab/balstilimab in KRAS-mut MSS mCRC: NCT06336902
- Agenus botensilimab/balstilimab expanded access: NCT06751524
- CCTG BATTMAN (botensilimab/balstilimab phase 3 vs BSC): NCT07152821
- DESTINY-CRC02 trial: NCT04744831
- ASCEND-CRC anti-EGFR rechallenge platform: NCT07318389
- EFFIPEC (CRS vs CRS plus HIPEC): NCT04861558
- University of Kentucky flat-dose vs weight-based MMC HIPEC: NCT04779554